



Discharge Summary

Name of the Patient	: Ram Bahadur	Payer	
Age	: 80	Date of Registration	
Gender	: Male	Date of Commencement	25/05/2026
UHID	: KABHK.0000277459	Date of Completion	02/06/2026
IPD number	: 3 HKIPDC14372	Type of Technique	CyberKnife
Admitting Doctor	Dr. Lohith G/Dr. Krithikaa S	Type of Summary	Completion Summary
Co-managed Doctor	Dr. Lohith G/Dr. Krithikaa S		

Department of Radiation Oncology

Final Diagnosis: Carcinoma prostate, adenocarcinoma Gleason score $3 + 4 = 7$, post surgery.

Was patient treated in HCG for the Disease: No

Brief History: The patient complained of burning micturition and fatigue and poor urine flow and abdominal discomfort in April 2023, for which further evaluation was done. PSA level on 17.04.2023 at Manipal Hospitals was 21.64 ng/mL. Following this, the patient did not receive any treatment. In 2026, he had the same recurring symptoms and was evaluated further for the same. MRI prostate done on 14.03.2026 at Isha Diagnostics shows prostate size $4 \times 2.6 \times 4.5$ cm, volume 23 cc. Peripheral zone is mildly compressed suggestive of changes of benign prostatic hyperplasia with ? prostatitis, PIRADS III score. Following this, the patient underwent TRUS biopsy on 20.03.2026 at Manipal Hospitals. Postop histopathology report dated 21.03.2026 at Manipal Hospitals showed acinar adenocarcinoma of the prostate, Gleason score $4 + 3 = 7$. Following this, the patient underwent PSMA PET/CT scan on 06.04.2026 at Newmedd Diagnostics which showed status post Foley procedure prostate gland along with cavity measuring $4.2 \times 4.1 \times 4.2$ cm tiny calcific foci noted adjacent to postoperative cavity. Diffuse tracer uptake noted in entire prostate gland with focal increase in left lateral aspect at the base. SUV 4.2. Following this, the patient has been referred for further management to Dr. Lohith G (Consultant Radiation Oncologist).

Past History: Not significant.

Personal History: Not significant.

Family History: Not significant.

Any history of missed treatment and lost for follow up: NA

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